

The 5 Topics That Trip Up 80% of CDIP Candidates

Free Study Breakdown by Sabeen (CDIP, CCS)

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Topic 1: SOI vs ROM — The Biggest Mistake

SOI (Severity of Illness): The impact of this illness on the patient's body RIGHT NOW. Changes hour to hour as patient improves or worsens.

ROM (Risk of Mortality): The statistical likelihood that this illness will kill the patient. Assigned at diagnosis based on the condition's inherent risk, NOT based on outcome.

The Trap Most Coders Fall Into

Outcome bias: "The patient recovered, so ROM must be lower."

Example: Septic shock patient arrives. Gets treated. Recovers and goes home. ROM still = Major (not Minor), because septic shock has a 30-50% mortality rate in the literature. The patient's good outcome proves the treatment worked—NOT that the risk was lower.

How to Get This Right

Assign ROM at admission based on the condition's inherent risk. Don't let the patient's outcome fool you. This distinction is worth 30+ points on the CDIP exam and thousands in DRG weight for the hospital.

Topic 2: Comorbidities Raise ROM, Not SOI

Key Insight: A 71-year-old with prior MI + CAD having routine post-op recovery = Moderate SOI + Major ROM. The hip fracture is moderate, but the patient's TOTAL

mortality risk is high due to age and cardiac history.

✗ Common Mistake

Thinking: "No acute cardiac event during this admission, so ROM is not elevated."

Reality: ROM reflects ALL risk factors (age, comorbidities, current illness), not just acute events. Medicare auditors expect ROM: Major in this case.

✓ Study Strategy

- When coding, ask: "What is the TOTAL mortality risk from this patient's condition(s) + age + comorbidities?"
- Don't sync SOI and ROM automatically. They can be very different.

📌 Topic 3: Good Outcomes ≠ Lower ROM

Fat Embolism Syndrome Example: Post-op day 3, patient develops FES. Hypoxic. Needs ICU. Recovers within 48 hours. ROM = Major (not Minor), because FES has a 10-15% mortality rate in the literature.

The Rule

ROM is assigned at diagnosis and reflects the condition's inherent risk, NOT the patient's individual outcome. Two FES patients—one recovers in 2 days, one progresses to ARDS and dies—both have ROM: Major because they both had the same condition.

Why This Matters

If you downcode ROM because a patient recovered, you're leaving money on the table. The hospital should be reimbursed for the RISK that was present, not just the outcome.

📌 Topic 4: Reading the Clinical Picture (The Query That's Worth \$10K+)

Real Case: Sepsis admission. Docs say: "Sepsis from UTI, treated and improved." But creatinine went 1.0 → 2.8. Query: "Is this AKI?" Answer: Yes. SOI jumps Moderate →

Major. DRG weight increases by \$12,000.

The CDI Job

- Read the clinical picture, not just the documentation
- Connect the dots (labs, trends, vital changes)
- Ask clarifying questions when severity isn't captured
- Understand DRG weight impact (why it matters)

✓ Study Tip

This is what separates good coders from great ones. It's not about knowing more codes—it's about asking the right questions.

Topic 5: Documentation Nuances Matter

Key Point: Vague documentation = lost revenue. "Sepsis, treated and improved" vs "Sepsis with acute kidney injury" = completely different DRG weights. Both patients might have had the same clinical picture, but documentation made the difference.

What to Look For

- Lab trends (creatinine, lactate, WBC)
- Medication patterns (vasopressors, antibiotics)
- Clinical events not mentioned in summary (intubation, dialysis)
- Organ dysfunction indicators

When to Query

When you see clinical evidence of severity that the docs didn't explicitly document, query for clarification. That one question can fund itself 10x over.

My Exact Study Strategy (How I Passed in 4 Months)

1. **Week 1-2: Master the 5 Topics Above** — Don't move on until SOI vs ROM is crystal clear. Do practice questions on just these topics until you're 90%+ consistent.

2. **Week 3-4: Learn Query Compliance Rules** — Study what you can and cannot query, AHIMA guidelines, physician communication best practices.
3. **Week 5-6: Practice Real Hospital Cases** — Work through de-identified case studies. Read charts. Identify severity. Write queries if needed.
4. **Week 7-8: Timed Practice Exams** — Take full 3-hour practice tests under timed conditions. Review every single wrong answer. Understand WHY, not just what the right answer was.
5. **Final Week: Review Only** — Don't learn new material. Review weak areas and take one final practice exam.

3 Practice Questions (Test Yourself)

Question 1:

A patient admitted with pneumonia. On day 2, develops acute kidney injury (creatinine 3.2). Treated with fluids and antibiotics. Discharged day 5 in stable condition.

What is the SOI and ROM?

- A) SOI: Major, ROM: Major
- B) SOI: Moderate, ROM: Minor
- C) SOI: Major, ROM: Moderate
- D) SOI: Moderate, ROM: Major

✓ Correct Answer: A (SOI: Major, ROM: Major)

The patient has two major severity drivers: pneumonia WITH acute kidney injury (organ failure). Both are major severity indicators. ROM is also Major because pneumonia with AKI carries significant mortality risk. Don't let the good discharge outcome fool you—ROM was Major at admission.

Question 2:

Patient admitted for routine appendectomy. History: Age 68, prior stroke, diabetes, hypertension. Post-op course unremarkable. Discharged day 2.

What is the SOI and ROM?

- A) SOI: Minor, ROM: Minor
- B) SOI: Moderate, ROM: Moderate
- C) SOI: Minor, ROM: Major
- D) SOI: Major, ROM: Major

✓ **Correct Answer: C (SOI: Minor, ROM: Major)**

The acute illness (appendectomy post-op) is minor—routine recovery. But the TOTAL mortality risk is major because of age (68) + prior stroke + multiple comorbidities. This is the classic "good outcome masks high mortality risk" scenario. SOI and ROM don't have to match.

Question 3:

A patient has sepsis from a UTI. Documentation says: "Sepsis treated, patient improved, discharged stable." Chart shows: Day 1 lactate 4.2, Day 2 lactate 2.1, Day 3 lactate 1.1. No mention of organ failure or vasopressor use.

Should you query? What should you ask?

- A) No query needed. Docs documented sepsis, that's sufficient.
- B) Yes, query: "Does this represent sepsis-induced organ failure?"
- C) Yes, query: "Why did the lactate improve?"
- D) No query needed. Improving lactate means no severe sepsis.

✓ **Correct Answer: B (Query about organ failure)**

The clinical picture (high lactate on admission) suggests significant sepsis. The documentation is vague about severity/organ involvement. A query asking about organ failure (shock, respiratory distress, renal involvement) is appropriate. This could elevate SOI from Moderate to Major, increasing DRG weight significantly. Don't assume lack of mention means it didn't happen—query for clarification.

✓ **Printable Study Checklist**

Before Taking the CDIP Exam, Master These:

- ☐ **SOI vs ROM Definitions** — Can you explain the difference in one sentence each?

- ☐ **Outcome Bias Trap** — Do you know 5 examples where good outcomes DON'T lower ROM?
- ☐ **Comorbidity Impact** — Can you ID cases where ROM is Major but SOI is Minor?
- ☐ **Reading Charts** — Can you spot clinical severity from labs/trends even if docs don't mention it?
- ☐ **Query Compliance** — Do you know AHIMA rules on when/how to query?
- ☐ **DRG Impact** — Can you explain why SOI/ROM matters for hospital reimbursement?
- ☐ **30+ Practice Questions** — Have you done at least 30 questions on SOI/ROM with 90%+ accuracy?
- ☐ **Full Practice Exam** — Have you taken at least one 3-hour timed practice test?
- ☐ **Weak Areas Review** — Have you gone back and reviewed every question you got wrong?
- ☐ **Query Examples** — Can you write a clinically appropriate, compliant query from a case study?

Ready to master CDIP and pass your exam?

This breakdown covers the top 5 topics. But there's much more.

The full 8-week course includes all 5 topics DEEP, 50+ practice questions, real hospital case studies, query compliance rules, and a study schedule that doesn't take over your life.

Coming soon from Sabeen (CDIP, CCS)